SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Pemetrexed Monotherapy

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Non-squamous NSCLC · standalone maintenance or second-line chemotherapy · NCCP 00222 · version 5 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pemetrexed is chemotherapy used to control selected non-squamous non-small-cell lung cancer that is locally advanced or has spread. Maintenance aims to keep cancer controlled after platinum chemotherapy has finished without the cancer progressing. Second-line treatment is a separate treatment decision after previous chemotherapy. Ask which aim and indication apply to you.

Schedule
One pemetrexed infusion on Day 1 of each 21-day cycle. Treatment is planned every three weeks while it is helping and side effects remain acceptable. There is no fixed total number of cycles in this regimen. Scans, symptoms, blood results, your priorities and the oncology review guide continuation, delays, changes or stopping.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pemetrexed
ANTIFOLATE CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Mouth soreness and diarrhoea
- ☐ Kidney problems
- ☐ Allergic or infusion reactions
- ☐ Skin rash; rarely serious skin reactions
- ☐ Rare serious lung inflammation

Discuss whether this is maintenance or second-line treatment, the individual aim, likely benefit and uncertainty, alternatives including not proceeding, common and serious harms, possible lasting kidney or nerve effects, fertility and pregnancy, vitamin and steroid medicines, practical burdens and your right to reconsider. A discussion aid supports but does not replace informed consent.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00222 v5.

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